Adoption Assistance Payments (AAP), Foster Care, Kinship-Guardian Assistance Program (KinGAP), and Unaccompanied Refugee Minor (URM)	Number	Page
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Revision Date:
September 1, 2022

Background:
Children who have been adopted from foster care, placed in foster care supervision, or unaccompanied refugee minors, may be eligible for Medi-Cal. This section provides policies to determine Medi-Cal eligibility for these children.

Purpose:
This section is updated to provide clarification regarding out of state non-Title IV-E Foster Care placements in California (CA).

Policy:
Basic Criteria:
To receive Medi-Cal under any aid code children must meet the following basic Medi-Cal criteria:

1. Meet institutional status requirements (Article 6 and Medi-Cal Eligibility Procedures Manual (MEPM) 6D).
   - Children placed at Napa State Hospital (a public mental institution under Welfare and Institutions Code (WIC) Sections 601 and 300) are under medical care, and not custodial care and may be eligible for Medi-Cal.
   - Custodial care means maintenance provided by a psychiatric institution to individuals whose condition does not warrant medical treatment.
   - Medical treatment includes psychotherapy, counseling, rehabilitation training, or other forms of medical treatment.

2. Not be under a penal hold. Because of the complexity of the penal system and the many types of penal holds certain clarifications are addressed below. They are not all inclusive and variations must be clarified on an individual basis.

Hold Type	Clarification
Under Arrest	Children, who have committed crimes, and who are detained and admitted to hospitals or held in Juvenile Hall (detained under WIC Section 602) prior to and/or after a detention order being signed, are under arrest. Medical treatment involving minor children requires legal consent of either the parent/guardian or some other agency or individual who has authority to give such consent. A law enforcement agency cannot obtain such authority without one of the following: • Placing the child under arrest • The parents formally surrendering their authority to the law enforcement agency • Court action.
Hold Pending Hearing, Arraignment, Trial, or Sentencing	Children released on bail are not considered “held” unless bail is revoked.

Serving a Sentence	• Children who are placed on parole or probation pursuant to a dispositional order under WIC Section 602 are not considered to be under a penal hold and may be eligible to Medi-Cal. They may be in Juvenile Hall awaiting placement in a Foster Care or group home or placed in a youth service program (for example, Rite of Passage or Boys Republic). • Children sentenced to serve terms under the California Youth Authority or sentenced to specific time to be spent in Honor Camps are under a penal hold and ineligible to Medi-Cal. • Children sentenced to a term of incarceration who are temporarily placed in a Foster Home pending available space in the detention center are ineligible to Medi-Cal.
Placement in Mental Institution Under a Pre-Trial or Pre-Sentencing Observation Order or Who Are Sentenced to Such an Institution	• Children placed at Camarillo or other State Hospitals must be evaluated to determine if they are still under a penal hold (ineligible) or have been placed on probation (potential eligibility). • Children discontinued from Medi-Cal under any program must be evaluated for eligibility under other Medi-Cal programs.

Adoption Assistance Payments (AAP):
Children may receive a federally funded adoption assistance subsidy under Title IV-E or a state-funded subsidy per state guidelines. Payments to the parents of an eligible child with special needs can take the form of either one-time (nonrecurring) or ongoing (recurring) adoption assistance.

Children receiving AAP payments are not always eligible to receive Medi-Cal if residing in California. AAP children from another state residing in California may receive Medi-Cal and must be assessed individually and, if eligible, granted under the appropriate Interstate Compact on Adoption and Medical Assistance (ICAMA) aid code. The dependent status of the child is not a factor in determining Medi-Cal eligibility.

Each child is in his/her own Medi-Cal Family Budget Unit (MFBU). The income and resources of the parents and siblings are not considered in the child's Medi-Cal eligibility determination.

AFDC-Foster Care (FC):
Children placed in foster care supervision may receive a federally funded payment under Title IV-E or a state-funded, non-Title IV-E payment per state guidelines. Some children might not be eligible for a payment, but might still be eligible for Medi-Cal.

Children placed under foster care supervision are not always eligible to receive Medi-Cal. Each case must be assessed individually and, if eligible, granted under the appropriate aid code. The dependent status of the child is not a factor in determining Medi-Cal eligibility.

Each child is in his/her own MFBU. The income and resources of the parents and siblings are not considered in the child's Medi-Cal eligibility determination.

Out of State Placements of Non-Title IV-E Foster Children in CA

Non-Title IV-E Foster children from out of state placed in a CA foster home, group home, residential facility or emergency shelter by another state agency are considered residents of the sending state and not eligible to Medi-Cal. Verify that the sending state is covering the child's Medicaid while in CA.

Non-foster care children from out of state placed in a CA medical and mental health facility, boarding school, educational institution, or other parent or legal guardian's home is considered a CA resident and potentially eligible to Medi-Cal.

Kinship-Guardian Assistance Program (KinGAP):

KinGAP establishes a separate payment rate for children who are leaving the foster care system and are placed with relatives who obtain guardianship.

KinGAP consists of two components – a federally-funded component and a state-funded component.

To qualify for the KinGAP Program, a child must have:

- Been removed from the parental home pursuant to a voluntary placement agreement or the result of a judicial determination to the effect that continuation in the home would be contrary to the welfare of the child and been adjudged a dependent child of the juvenile court pursuant to WIC Section 300, or a ward of the juvenile court pursuant to WIC Section 601 or 602
- Been residing for at least 6 consecutive months in the approved home of the relative while under the jurisdiction of the juvenile court or under a voluntary placement agreement or of a previously approved relative guardian
- A written binding agreement entered into by the relative guardian and the county welfare agency, probation department, or Title IV-E tribal agency, prior to the establishment of the guardianship
- A kinship guardianship established pursuant to WIC Sections 366.26, 360, or 728(d), and dependency dismissed pursuant to WIC Section 366.3, or wardship terminated pursuant to WIC Section 738(e) concurrently or subsequently to the establishment of the kinship guardianship.

Each KinGAP child is in his/her own MFBU, even if there are other siblings or a needy caretaker relative in the home.

Unaccompanied Refugee Minor (URM):

The Department of Health Care Services (DHCS) provides full scope Medi-Cal without a share of cost to children in the URM Program, if otherwise eligible. The URM Program provides culturally and linguistically appropriate child welfare, foster care, and independent living services to minors who do not have parents in the United States (U.S.), or who enter the U.S. unaccompanied by a parent, an immediate adult relative, or an adult having documentable legal evidence of custody of the minor. These children are placed into the URM Program by the Office of Refugee Resettlement (ORR). URM children do not enter the state's child welfare system because they are not usually removed from the home of a parent or legal guardian due to abuse, neglect, or through a voluntary agreement.

For Medi-Cal purposes, URM children are treated like foster care children. Unlike foster care regulations, ORR regulations do not require that URM children be placed in the care and custody of the state. ORR regulations require states to ensure legal responsibility, including legal custody and/or

guardianship, is established for each URM child. Currently the state has three URM Program service providers:

- Catholic Charities of Santa Clara County (CCSCC)
- Crittenton Services for Children and Families (Crittenton) in Orange County
- International Christian Adoptions (ICA) in Sacramento County

These service providers may place youth who are in the federal URM Program with certified foster parents in neighboring counties.

Procedure:
Follow the rules above to determine eligibility for Medi-Cal for children in these programs.

Program Impacts:
Other Programs Affected
Foster Care

References:
Medi-Cal Eligibility Procedures Manual (MEPM) 6A
All County Welfare Directors Letters (ACWDLs) 11-06, 18-14, and 21-28

Sunset Date:
This policy will be reviewed for continuance by September 30, 2025.

Approval for Release:



Rick Wanne, Director
Self-Sufficiency Services

Revision Date:
October 1, 2018

Background:
An unaccompanied refugee minor may be eligible for Medi-Cal. This section provides the policies to determine Medi-Cal eligibility for these children.

Purpose:
This section is added for the Unaccompanied Refugee Minor (URM) requirements.

Policy:
URM Application Process:
Social Workers (SWs) from the URM service providers submit Medi-Cal applications to the County on behalf of children placed with their agency in the URM Program. Typically, Medi-Cal applications that are mailed in will be accompanied by a coversheet that identifies the individual applying as a representative of the URM service provider. In addition, the application must include an approval letter from ORR to confirm that the child is a federal URM Program participant, if the child is in any of the following immigration statuses:

- Asylee
- Cuban/Haitian entrant
- Victim of human trafficking (T visa)
- Non-citizen victims of domestic violence and other serious crimes who have been granted a U visa
- Youth who have been granted Special Immigration Juvenile Status

The ORR does not provide URM youth who are in refugee status with an approval letter. The SW will provide a verification letter from CDSS as proof of URM Program eligibility for refugees.

Because URM youth are to be treated like foster care children for Medi-Cal eligibility purposes, the Medi-Cal application must include the following:

- A completed MC 250/MC 250A form
- A copy of the URM Designation letter from ORR or URM Approval letter from CDSS

Other information needed for the case record includes:

- A SSN
  (Note: Do not delay or deny services for otherwise eligible URM youth who fail to furnish their SSN. If an applicant has not been issued a SSN, normal rules for obtaining a SSN apply. Update the MEDS record when SSN information is provided.)

- Immigration status information and/or an immigration document
  (Note: Do not delay or deny services for otherwise eligible URM youth who fail to furnish their immigration status. Update the MEDS record when immigration status is provided.)

Medi-Cal Eligibility and Immigration Status Information:
All eligible children in California under the age of 19 receive full scope Medi-Cal regardless of immigration status, but verification of immigration status is still required for children with satisfactory

immigration status (SIS) because State law requires the department to maximize federal financial participation to the fullest extent possible.

Verify the immigration status of URM youth using the Systematic Alien Verification for Entitlements (SAVE) system unless the Federal Data Services Hub, via the California Healthcare Eligibility, Enrollment and Retention System (CalHEERS), has already verified their status. Immigration status information or an immigration document for the URM youth will be provided along with the URM designee letter when available.

Youth in the URM Program are eligible to full scope Medi-Cal regardless of immigration status. If immigration status information is not available at the time of application, do not delay establishing eligibility during the verification process if the URM youth is otherwise eligible.

Nevertheless, if the immigration status of a URM applicant cannot be verified at the time of application, follow up with the service provider. Verify the immigration status of URM youth in accordance with policy.

County of Responsibility:
For purposes of Medi-Cal eligibility, the county of responsibility for URM youth is the county in which the youth resides. Therefore, the instructions on the back of the MC 250 form do not apply to URM cases.

Foster Care Maintenance Payments:
Youth in the URM Program do not receive assistance payments through the counties. The only benefits URM youth will access through the county is Medi-Cal under aid code 45 or 4M. All other services and benefits are provided through the URM service providers. These URM services are funded through ORR, through CDSS administered contracts with the three URM Program service providers.

Extended Foster Care:
URM youth are eligible for the URM version of extended foster care if they are between the ages of 18 and 21, and meet all eligibility requirements. URM youth who do not meet extended foster care eligibility requirements should be placed into aid code 45 for Medi-Cal purposes.

Former Foster Youth:
URM youth who meet Former Foster Youth (FFY) Program eligibility requirements are eligible to continue receiving full scope Medi-Cal under the FFY Program. When the worker learns that a FFY eligible URM youth has exited their URM placement (at age 18 or older), the worker will transition the youth into the Medi-Cal program for FFY and assign the 4M aid code even if the youth's whereabouts are unknown. When URM youth age out of the FFY Program at age 26, follow the FFY age out process.

Annual Redetermination:
Follow Medi-Cal Foster Care and FFY annual redetermination rules for the URM youth. At annual redetermination, contact the URM service provider for individuals who are not in the FFY Program.

This includes:

- URM Program certified youth who are under 18 years of age and receiving Medi-Cal benefits under aid code 45
- URM youth 18 through 20 years of age who have elected participation in extended foster care placement as designated by the service provider.

The service provider must verify URM Program eligibility status prior to complete an annual redetermination.

When the worker is notified of a change in circumstances resulting in a youth exiting URM foster care placement between 18 and 21 years of age, or when the URM youth turns 21 years of age, use the MC 250A form and transition the youth into aid code 4M.

Upon transition into aid code 4M, send out the Notice of Action (NOA) MC 239 FFY-4, informing the beneficiary of continued Medi-Cal coverage. Also use this NOA to inform youth exiting foster care at age 18, or older, that they have been automatically moved into the FFY Program.

Once the URM youth is transitioned into aid code 4M, send the NOA MC 239 FFY-3 at annual redetermination, advising them of their continued enrollment in the FFY Program. Even if the NOA is returned to the County due to loss of contact, the youth must be left in 4M, until age 26. When there is a loss of contact with a 4M beneficiary, move the youth into fee-for-service Medi-Cal if they are in a managed care plan. Because there is no MEDS transaction for workers to move a beneficiary from managed care to fee-for-service Medi-Cal, submit these requests to the Medi-Cal Managed Care Division Office of the Ombudsman.

Do not terminate eligibility for aid codes 45 or 4M provided to URM youth prior to age 26 unless it is verified that the youth is deceased or not a California resident.

If a URM youth is a Medi-Cal beneficiary on the date he/she becomes an inmate of a public institution, Medi-Cal benefits are suspended; not terminated.

URM Beneficiaries and Managed Care:

The Medi-Cal provided the URM youth is fee-for-service Medi-Cal or optional managed care. This means that URM youth in non-County Organized Health System counties, are in fee-for-service Medi-Cal unless they opt into a managed care plan. URM beneficiaries receiving fee-for-service Medi-Cal will not receive a Health Care Options (HCO) packet unless one is requested. If the individual chooses to opt into a managed care plan, he/she will need to select a health care provider. To join a managed care plan, or to get more information about a managed care plan, direct beneficiaries or the service providers to contact HCO at 1-800-430-4263 or Health San Diego at 619-515-6584.

Inter-County Transfers:

Follow Medi-Cal Inter-County Transfer (ICT) rules for URM beneficiaries. **Do not** use the Foster Care ICT process for the URM population. Medi-Cal URM youth or their service provider must promptly notify the County of any change in circumstances including a change in county of residence. Upon notification of a change in county of residence, initiate an ICT. The original county of residence must

update MEDS to reflect necessary changes to the Current Residence Address and Res-County date fields.

Do not require the URM service provider to submit a new application for Medi-Cal in the new county of residence for a URM beneficiary who is already on Medi-Cal. The case will remain active in the sending county until the receiving county has taken over the record. Counties must work together to ensure there is no break in coverage or benefits during the transition from one county to another.

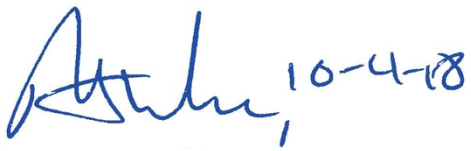
Procedure:
Follow the rules above to determine eligibility for Medi-Cal for children in these programs.

Program Impacts:
Other Programs Affected
Foster Care

References:
All County Welfare Directors Letter (ACWDL) 18-14

Sunset Date:
This policy will be reviewed for continuance by 9/30/2021.

Approval for Release:



Rick Wanne, Director
Eligibility Operations

Revision Date:
September 1, 2020

Background:
This section defines the various aid codes, when they are used, and the procedures to follow.

Purpose:
This section is updated to revise the Kin-GAP aid codes.

Policy:
Federally Eligible Adoptions Assistance Program (AAP):
Use the following aid codes for federally eligible AAP children:

<table>
<tr><th>Aid Code</th><th>Description</th></tr>
<tr><td>03</td><td>Used for federally eligible Title IV-E AAP children placed by the State of California. They are eligible for Medicaid but may not be eligible for Medi-Cal depending on where they reside. Consider Puerto Rico, Guam, and other U.S. territories as other states to determine residence.

<table>
<tr><th>If the child is placed…</th><th>Then…</th></tr>
<tr><td>In California or in another country by California,</td><td>Aid Code 03 will automatically issue a Medi-Cal Beneficiary Identification Card (BIC).</td></tr>
<tr><td>In another state by California,</td><td>Child Welfare Services (CWS) is responsible for arranging Medicaid coverage for these children. The Medi-Cal worker must send a timely notice discontinuing Medi-Cal.</td></tr>
</table>

Note: Requests may be received from either the placing state or the Adoption Social Worker.</td></tr>
<tr><td>06</td><td>Used for children receiving federal Title IV-E AAP cash subsidy from outside California through the Interstate Compact on Adoption and Medical Assistance (ICAMA). This aid code is full-scope zero share of cost (SOC) Medi-Cal.

The following conditions apply:
• This is a mandatory categorically eligible population. Eligibility for Medi-Cal has been met through receipt of Title IV-E AAP benefits.
• No separate application form is required to establish Medi-Cal.
• No income or asset test is required.
• Enrollment in managed care plans is voluntary.
• Annual redetermination is conducted with the adoption state, not the adoptive parents.
• Inter-County Transfer procedures apply if the child moves to a different county within California.
• Children are eligible to Continuous Eligibility for Children (CEC) if they are no longer</td></tr>
</table>

	eligible under AAP prior to their 18th birthday and if they age out of the AAP program at age 18 years.
07	Used for ongoing adoptive placement of hard-to-place non-minors, whose initial AAP payment occurred on, or after age 16, and who are over age 18, but under age 21, who would require placement without such assistance. Note: Medi-Cal is automatic with the determination of eligibility for the IV-E payment.

Non-Federally Eligible AAP:
Use Aid Code 04 for non-federally eligible AAP children placed by California. These children are eligible to Medicaid regardless of where they reside.

If the child is placed…	Then…
In California or in another country by California	These children automatically receive a Medi-Cal BIC through the 04 case.
In a Non-ICAMA state by California,	California does not provide Medi-Cal for these children. They must receive Medicaid from the state of residence.

Use Aid Code 4A for non-federally eligible AAP children placed in California by another ICAMA state and there is a state-only AAP agreement between that state and the adoptive parents. This aid code is full-scope, zero SOC Medi-Cal and enrollment in managed care plans is voluntary.

Federally Eligible AFDC-Foster Care (FC):
Use the following aid codes for FC children who are federally eligible to AFDC-FC:

Aid Code	Description
42	Used for federally eligible children placed in AFDC-FC by California agencies. Consider Puerto Rico, Guam, and other U.S. territories as states to determine residence for this aid code.
46	Used for Title IV-E federally funded children placed in California from another state through the Interstate Compact for the Placement of Children (ICPC). This aid code is full-scope zero SOC Medi-Cal. The following conditions apply: • This is a mandatory, categorically eligible, population. Eligibility for Medi-Cal has been met through receipt of Title IV-E FC benefits. • No separate application form is required to establish Medi-Cal. • No income or asset test is required. • Enrollment in managed care plans is voluntary. • Annual redetermination is conducted with the placement state, not the foster parents. • Inter-County Transfer procedures apply if the child moves to a different county within California. • Children are eligible to CEC if they are no longer eligible to foster care prior to their

	18th birthday. • Children are eligible to the FFY Program, aid code 4M, upon aging out of the foster care system.
49	Covers non-minor dependents (NMDs), age 18 but under age 21, on whose behalf federal AFDC-FC financial assistance is provided for FC placement. Note: Medi-Cal is automatic upon the determination of eligibility for the Title IV-E payment.

Non-Federally Eligible AFDC-FC:
Use the following aid codes for FC children who are non-federally eligible to AFDC-FC.

Aid Code	Description
40	Used for non-federally eligible children placed in FC by California. Note: This aid code includes children who might be otherwise federally eligible, but who are placed in "for-profit" facilities, such as Rite of Passage or Boys Republic. Assign the correct aid code so that children placed in "for-profit" facilities out-of-state will continue to receive their Medi-Cal.
43	Covers NMDs, age 18 but under age 21, on whose behalf state-only AFDC-FC financial assistance is provided for foster care placement. Note: These youth must meet the eligibility requirements for the Former Foster Youth (FFY) Program.

Foster Care Medi-Cal Only:
Use Aid Code 45 for children placed in FC out of the home when a public agency is assuming financial responsibility for the child's care and the agency has responsibility for the child's placement, including:

- Children receiving aid under the CalWORKs (CW) Aid Codes (FC payments made through County Treasurer Funds) when they meet the basic criteria
- Children placed in the County Mental Hospital (CMH) when public funds are used for their personal needs

Use this aid code also when a Medi-Cal only case is opened to meet the immediate medical need of a child who has not been placed, or whose AFDC-FC eligibility cannot be established, and when the child is not currently active in a CalWORKs or no SOC Medi-Cal case. Refer to MPG 5.11.04 for BIC issuance instructions.

Clarification: Children at Door of Hope are ineligible under this aid code unless they are receiving public funds. Their needs are usually met by Door of Hope, which are private funds. These children may be eligible under Aid Codes 82/83 or 34/37.

The following forms are required to establish eligibility:

Form	Description
SAWS 1	Written request for aid
MC 250 Application form	Completed by a representative of the public agency assuming financial responsibility.

KinGAP:
Use the following aid codes for KinGAP children:

Aid Code	Description
4F	KinGAP cash assistance. Covers children up to 17 years of age inclusive in the Kin-GAP non-federal program for children in relative guardian placement receiving cash assistance.
4G	Children in KinGAP relative placement receiving cash assistance with no federal financial participation (FFP) cash payments. This is a state-only cash assistance program. Serves youth ages 18 through 20 years old inclusive due to a disability. The Aid Code is full-scope zero SOC Medi-Cal. Enrollment in managed care plans is voluntary.
4S	Covers former and current FC NMDs, over age 18, but under 21, by moving them from FC placements to more permanent placement options through the establishment of a relative guardianship that occurred on, or after, age 16. (also includes youth age 18 but under 21 based on a disability.) Note: Medi-Cal is automatic upon the determination for the Title IV-E payment.
4T	Children in KinGAP relative placement receiving federally funded KinGAP. The Aid Code is full-scope zero SOC Medi-Cal. Enrollment in managed care plans is voluntary.
4W	KinGAP. Covers NMDs, aged 18 through 20 inclusive.

URM:
Use Aid Code 45 or 4M for URM youth as stated above since these children do not receive aid payments from the county.

MAGI Medi-Cal:
Use the appropriate MAGI Aid Code for children (in or under FC supervision and others) who meet the linkage requirements for MAGI Medi-Cal. See the MAGI Medi-Cal sections for more information on requirements for this program.

Medically Indigent:
Use Aid Codes 82/83 for children (in or under FC supervision and others) who do not meet CalWORKs linkage requirements.
This category includes, but is not limited to:

- Children, not living in the home of a parent or relative, who are in a medical facility where a public agency is not assuming financial responsibility because the parent(s) is/are providing all personal needs. This includes state hospitals such as Camarillo when the parent(s)/relative(s) is/are providing for the child's personal needs.
- Children placed with non-relatives where the child's own income exceeds the FC rate.
- Children placed back with the parent(s) under FC supervision and there is no CalWORKs linkage.
- Children placed in Long Term Care (LTC) when no public agency is providing for personal needs wholly or in part.
- Seriously Emotionally Disabled (SED) children without CalWORKs linkage for whom disability has not been established. These children are determined to be "living in the home" and only temporarily absent for educational reasons.

The following forms and are required to establish eligibility:

Form	Description
SAWS 1	Written request for aid
FC 2	• Foster Care Statement of Facts • The representative of the agency having legal responsibility for the child must complete the FC 2 • HHSA has legal responsibility for the child in a dependence situation • The person who has the most information should complete the FC 2 (Social Worker or worker)

Medically Needy:
Use Aid Codes 34/37 for children and adults who are CalWORKs linked (meet a deprivation factor).

This category includes, but is not limited to:
- Children placed back in the parent's home under FC supervision, and where the child is linked to CalWORKs.
- Children placed in the home of a relative who refuses, or is ineligible to, AFDC-FC payments.
- SED children when CalWORKs linkage exists.

The same forms and verifications for Medically Indigent are required for Medically Needy.

Foster Care CalWORKs:
Use the following aid codes for children ineligible to AFDC-FC and receiving CalWORKs.

Aid Code	Description
4H	For children receiving CalWORKs.
4N	Covers NMDs, aged 18 but under 21, on whose behalf CalWORKs financial assistance is provided for FC placement, and who is living with an approved relative. Note: These youth must meet the requirements of the FFY Program.

Procedure:
Follow the rules above to determine the aid codes for these children.

Program Impacts:
Other Programs Affected
Foster Care

References:
Title 22, Sections 50205 and 50251
Medi-Cal Eligibility Procedure Manual 5E
All County Welfare Directors Letters (ACWDLs) 00-22, 08-30, 08-30E, 10-28, 10-28E, 11-06, 12-03, 12-03E, 12-03EE, 12-12, and 18-14

Sunset Date:
This policy will be reviewed for continuance by August 31, 2023.

Approval for Release:

9-10-20

Rick Wanne, Director
Eligibility Operations

Revision Date:
October 1, 2018

Background:
This section provides instructions regarding the issuance of the BICs for children in FC and AAP aid codes.

Purpose:
This section is re-written in the new format.

Policy:
Polinsky Children's Center (PCC):
The PCC worker will clear the child on the MEDS INXB Screen to determine if a BIC has ever been issued for the child.

If…	Then…
No BIC issued,	Initiate the case using the address where the child is residing or is going to reside. If unknown, use the PCC address.
BIC issued on active CalWORKs, Aid Code 38, or no share of cost (SOC) Medi-Cal,	a. Complete form KM80 including the: • CIN • Child's date of birth • BIC issue date • OHC information b. Place one copy of the KM80 in the child's medical file, which is given to the foster parent. c. Complete a request to disenroll from a Pre-Paid Health Plan (PHP) if the child is in a PHP Note: At the request of the foster parent, the child may remain in the PHP.
BIC issued on inactive CalWORKs, Medi-Cal, or active SOC Medi-Cal,	a. Initiate the case using the address where the child is residing or is going to reside. If unknown, use the PCC address. b. Complete the request to disenroll from a PHP if the child is in a PHP, unless the foster parent requests that the child remain in the PHP as noted above.

Probation:
Follow the following procedures when providing Medi-Cal for wards after their Disposition Hearing.

Step	Action
1	Obtain the Medi-Cal application for the child, completed by the Probation Department.
2	Clear the child on MEDS INXB Screen to determine if a BIC has ever been issued for the child.
3	Initiate the case using the following address: Prob Dept Plcmt Supver

	PO Box 23596 San Diego, CA 92139-2596
4	Request a replacement BIC only if the Probation Department requests a BIC and a BIC has previously been issued.

All Other Placements:
Follow the procedures below when providing Medi-Cal for children in placements other than PCC and Probation.

The FC worker will clear the child on the MEDS INXB screen to determine if a BIC has ever been issued for the child.

If…	Then…
No BIC was issued,	• Initiate the payment case • Notify the foster parent on the Notice of Action (NOA) if the child has Other Health Coverage (OHC)
BIC was issued,	Notify the parent on the NOA if the child has OHC.

Removing OHC Indicator:
To prevent barriers to medical services for FC children, DHCS will remove the OHC code on MEDS at the request of a duly authorized party such as:
- A worker
- A FC worker
- An enrolled Medi-Cal provider of service.

Staff may request the removal of the OHC indicator if they have either written documentation, or documentation of oral communication from the other health insurer, which confirms that the specific provider, service, service frequency, or location is not covered by the OHC. If it is within fifteen days of a written or oral request made by a duly authorized party for confirmation of non-coverage, the other health carrier does not provide a written response or an oral response that can be documented, DHCS will grant the request to remove the OHC indicator. Image all documentation of non-coverage in the FC child's case.

Request the removal of the OHC indicator via the Ombudsman's website.

Detention Orders – Immediate Need:
Treat children entering the FC system as immediate need, as soon as the detention order is issued. The FC worker will receive the necessary information from the Social Worker (SW) to establish Medi-Cal eligibility for the child.

In addition:
- Staff in the Court Unit will send the detention calendars, on a daily basis to FC staff. The detention calendars list children who will have a detention hearing on that day. Clear each child on MEDS, to identify if he/she has a MEDS record.
- Children with active MEDS require no further action.

- Children who are not active on MEDS will require expedited Medi-Cal handling if they are in "out of home" care.
- An expedited Medi-Cal Request Referral Form will be faxed to the SW's supervisor (or duty worker of the day) requesting an MC 250, SAWS 1, and MC 13, to be faxed back within 24 hours for children in placement.
- Establish Medi-Cal eligibility in CalWIN, upon receipt of the forms, for children who are not already Medi-Cal beneficiaries. The FC worker will submit an on-line entry request to the MEDS clerk to immediately enter the Medi-Cal eligibility.
- Record the BIC number on the on-line entry request upon completion of the on-line entry and return to the FC worker.
- Issue Proof of Medi-Cal eligibility for children without a previous Medi-Cal record.
- Issue Proof of Medi-Cal eligibility at the request of a child's authorized representative for any child who has active Medi-Cal at the time of removal from the home, but who does not have access to his/her BIC. Authorized representatives include the SW or substitute care provider.
- Proof of Medi-Cal eligibility includes:
    - A paper Medi-Cal BIC
    - The BIC number from the MEDS record

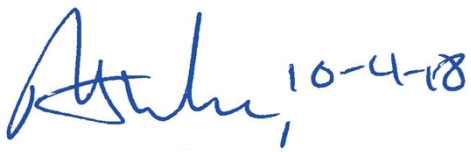
Procedure:
Follow the rules above to determine eligibility for Medi-Cal for children in these programs.

Program Impacts:
Other Programs Affected
Foster Care

References:
All County Welfare Directors Letters (ACWDLs) 93-53, 94-05, 94-25, 94-28, 94-47, 94-51, 01-41, and 01-61

Sunset Date:
This policy will be reviewed for continuance by 9/30/2021.

Approval for Release:



Rick Wanne, Director
Eligibility Operations

Retroactive Benefits	Number	Page
	5.11.05	1 of 1

Revision Date:
October 1, 2018

Background:
This section provides instructions regarding retroactive Medi-Cal coverage for children in FC and AAP.

Purpose:
This section is re-written in the new format.

Policy:
When retroactive Medi-Cal is requested for a child placed in California by another state, obtain the following information, if not already obtained, from the placing state:

- Date the child moved to California
- Written verification that the child is federally eligible for AAP or AFDC-FC. The statement must include the following:
  - Name of child and of the adoptive/foster parent
  - Child's SSN, date of birth, and address
  - Certification of Title IV-E federal eligibility to AAP/AFDC-FC
  - Termination date of Medicaid benefits from the placing state
  - OHC
  - For AAP cases, the adoption agreement termination/renewal date and a statement that the adoption agreement is on file with the placing state.
  - For AFDC-FC cases, the name of the agency in the placing state with responsibility for care and custody of the child and verification that the child is receiving an AFDC-FC grant.

Do not grant retroactive Medi-Cal prior to the month the child moved to California, or three months prior to the month the MC 250 is dated, whichever is later.

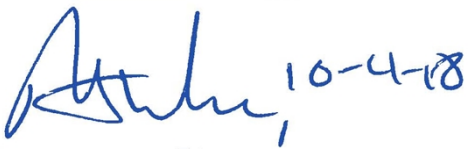
Procedure:
Follow the rules above to determine eligibility for Medi-Cal for children in these programs.

Program Impacts:
Other Programs Affected
Foster Care

References:
Title 22, Sections 50148, 50193, and 50197

Sunset Date:
This policy will be reviewed for continuance by 9/30/2021.

Approval for Release:



Rick Wanne, Director
Eligibility Operations